SACTCHECK · REGIMEN-SPECIFIC SACT CONSENT

Atezolizumab + carboplatin + etoposide

PROTOTYPE · GOVERNANCE REVIEW REQUIRED

First-line extensive-stage small-cell lung cancer · NCCP 00689 · version 4 · 21-day cycle

Patient name	DOB	Hospital no.	Date

Treatment context
To control extensive-stage small-cell lung cancer, relieve symptoms and help people live longer. The balance of benefit, side effects and time in treatment should be reviewed with you.

Schedule
Induction: atezolizumab and carboplatin on Day 1, with etoposide on Days 1–3, repeated every 21 days for up to four cycles. Maintenance: carboplatin and etoposide stop; atezolizumab alone continues every 21 days while appropriate. The fourth cycle ends about 12 weeks after starting if there are no delays; maintenance has no fixed total duration in this regimen.

1. TREATMENT DISCUSSION

- ☐ Purpose, expected benefit and alternatives explained
- ☐ Indication, medicines, sequence and duration confirmed
- ☐ Blood tests and any individually indicated heart checks discussed
- ☐ Support medicines and phase changes explained
- ☐ Fertility, contraception and breastfeeding discussed where relevant
- ☐ Common, serious and lasting drug-specific risks discussed below
- ☐ Urgent symptoms and local contact details confirmed

2. REGIMEN / AGENT-SPECIFIC MATERIAL RISKS

Atezolizumab
IMMUNOTHERAPY

- ☐ Immune inflammation of lungs, bowel, liver, kidneys, hormone glands, skin, heart, muscles or nerves
- ☐ Serious infusion or SC-related reactions
- ☐ Some effects require prolonged treatment or lasting hormone replacement

Carboplatin
CHEMOTHERAPY

- ☐ Low blood counts, infection, anaemia and bleeding
- ☐ Nausea, vomiting and fatigue
- ☐ Allergic reactions; renal function affects dosing

Etoposide
CHEMOTHERAPY

- ☐ Low blood counts and infection
- ☐ Hair loss, mouth soreness, nausea and bowel changes
- ☐ Infusion reactions or low blood pressure during administration
- ☐ Possible later treatment-related leukaemia; discuss longer-term risks

Discuss the aim of disease control and symptom relief, expected benefits, alternatives including a different systemic plan or supportive care, and the option to revisit decisions. Include immune effects that can be serious, delayed or require long-term hormone replacement, chemotherapy infection/bleeding risks, hair loss, fatigue and practical route choices. Rare serious heart, muscle and nerve immune effects can overlap and need urgent assessment.

Individual aim / benefit, alternatives, material risks and questions:

3. CONSENT CONFIRMATION

- ☐ Expected benefit, important risks and reasonable alternatives discussed
- ☐ Patient questions invited and answered; patient-specific material risks / priorities considered
- ☐ Emergency contact / toxicity advice and written information offered where appropriate
- ☐ Patient understands consent is voluntary and may be withdrawn

Patient / person giving consent _____ Date _____

Clinician obtaining consent _____ Date _____

Prototype only. This one-page form is a regimen-specific consent aid and requires formal governance, consultant and oncology-pharmacy review before clinical use. The current NCCP regimen, local/national consent policy, current medicine information, patient-specific material risks and clinical judgement remain authoritative.
Base regimen: NCCP 00689 v4.

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Developed by Dr Paul O'Brien